

B10 44E3. 24 NOV 2025

EMERGING. new.

Zoonotic. spillovers (?)
($R_0 < 1$)

[could also refer.
expanding its range
becoming more infectious
more virulent
antibiotic resistant]

RE

RE-EMERGING DISEASE.

Previously controlled but
is resurging

(socioeconomic change,
disruption of society, ...)

biology, landscape change,
climate change, socioeconomic change

ENCOUNTER + COMPATIBILITY filters
(landscape etc.) (biological)

• DISEASE TRIANGLE.

host, parasite, environment
(plant pathology).

LOCAL > GLOBAL

global change can ~~not~~ be good or
bad for disease

• MALARIA in NORTH AMERICA.

- brought to the 'New World' by
European colonists
- especially by west African
enslaved people.

well established the 1850s.

poorly diagnosed, poorly treated.

DECLINED 1900 - 1920 (drainage)

1930s. 5000 deaths/year (US)

1950s. < 10 deaths/year

Improved NUTRITION, medical care,
housing, treatment.

R_0 for vector-borne diseases: pop.
PROPORTIONAL to vector density

MALARIA + local landscape change:

- western Kenya:
malaria prevalence increased
 $\Rightarrow$ due to availability
? of brick-making pits

- VECTOR POP DYNAMICS:
temperature
hydrology
• predators.

• DEFORESTATION in Amazon.

increase malaria prevalence
(due to forest edge habitat)
increased human-vector
contact

$\hookrightarrow$ MALARIA decreases deforestation.

- mosquito-borne disease in
temperate countries.

- expansion of vector ranges?

~~Local~~

spillover $\rightarrow$ Local transmission
(hybrids get events
infected from animals) autochthonous.

$R_0 > 0$

✓ MALARIA. Anopheles mosquitoes.

✓ chikungunya, Zika, dengue, ✓
yellow fever, ... Aedes.

Aedes aegypti. A. albopictus.

climate change on malaria range?

• Indian tea plantations.

• what's the actual change in temp?

temp changes:

Anopheles $\rightarrow$ Aedes.

malaria $\rightarrow$ dengue,
yellow fever,
chikungunya.